

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction for the administration of Dexamethasone/Neomycin sulfate/Acetic acid 0.1%w/w / 0.5%w/w / 2%w/w Ear Spray for the treatment of Otitis Externa

Summary of NICE CKS Guidelines for Otitis Externa

Overview

- **Otitis externa** is **inflammation of the external auditory canal**, often due to infection.
- Common in swimmers and people with excessive ear cleaning or hearing aids.
- Can be **acute**, **chronic** (lasting >3 months), or **malignant/necrotising** (a rare but serious form).

Symptoms

- Ear pain (otalgia)
- Itching
- Discharge (otorrhoea)
- Hearing loss (due to canal swelling or blockage)
- Tenderness of tragus/pinna
- Swollen, erythematous external canal

Assessment

Diagnosis

- Clinical diagnosis based on:
 - Symptoms and signs of canal inflammation
 - Exclusion of other causes (e.g. otitis media, foreign body)

Red Flags – Urgent Referral

- **Severe unremitting pain**, especially in diabetics or immunocompromised (suspect **malignant otitis externa**)
- **Facial nerve palsy**

- **Systemic illness or spreading infection**
 - **Failed treatment after 2 weeks**
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Management

General Measures

- Advise **no water in the ear** during treatment (avoid swimming).
- **Avoid cotton buds**, earplugs, and self-cleaning.
- Use analgesia (e.g. paracetamol or ibuprofen) for pain relief.

Topical Treatment (First-Line)

- **Topical antibiotic +/- steroid ear drops:**
 - **First-line: Dexamethasone with neomycin**
 - **Alternative: Acetic acid 2%** (antiseptic – for mild cases)
- **Duration:** 7 days, extend to 14 days if needed.

Ear drops must be **administered correctly**: keep head tilted or lie on the side for a few minutes after applying.

Aural Toilet

- Gently clean the canal if discharge or debris is obstructing the drops.
 - Performed by trained professionals (e.g. ENT or practice nurse).
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Chronic Otitis Externa

- May be allergic, dermatological (e.g. eczema), or fungal.
 - Avoid long-term antibiotics.
 - Consider **referral to ENT** for recurrent or chronic cases.
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Fungal Otitis Externa

- Suspect if symptoms persist despite antibiotics or if white/black debris visible.
 - Treat with **antifungal ear drops** (e.g. clotrimazole) or ENT referral.
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Malignant Otitis Externa (Necrotising)

- **Emergency referral** to ENT.
- Most common in **elderly diabetics or immunocompromised patients**.

- Requires IV antibiotics and imaging (e.g. CT or MRI).
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Follow-Up

- Review if symptoms worsen or don't improve within 7 days.
- Refer if:
 - o Recurrent episodes
 - o Uncertain diagnosis
 - o Suspected complications or resistant organisms

Patient Group Direction

for the supply/administration of

Dexamethasone/Neomycin sulfate/Acetic acid 0.1%w/w /
0.5%w/w / 2%w/w Ear Spray

for the treatment of

Otitis Externa

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Dexamethasone/Neomycin sulfate/Acetic acid 0.1%w/w / 0.5%w/w / 2%w/w Ear Spray is indicated for the treatment of mild to moderate acute otitis externa in adults and children aged 2 years and over.
Inclusion criteria	- Adults and children aged 2 years and over. - Presenting with clinical signs and symptoms of acute otitis externa. - Informed consent obtained from patient or guardian. - No contraindications to Dexamethasone/Neomycin sulfate/Acetic acid 0.1%w/w / 0.5%w/w / 2%w/w Ear Spray.
Exclusion criteria	- Known hypersensitivity to neomycin, dexamethasone, acetic acid, or other excipients. - Suspected or known tympanic membrane perforation. - Where a tympanostomy tube (grommet) is in situ - Otitis media or systemic infection requiring oral antibiotics. - Dexamethasone/ Neomycin sulfate/ Acetic acid ear spray is not recommended during pregnancy. - Children under 2 years of age.
Cautions	- Ensure tympanic membrane is intact before use. - Avoid prolonged use to reduce risk of sensitisation and resistance. - Advise patient to shake bottle well before use and avoid water entering ears. - Refer if no improvement within 7 days, if symptoms worsen or if irritation or rash occurs. - A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from Dexamethasone/ Neomycin sulfate/ Acetic acid ear spray therapy, taking into account the benefit of breast-feeding for the child and the benefit of therapy for the woman. A risk to the breast-fed child cannot be excluded. - Visual disturbances have been reported with corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Dexamethasone/Neomycin sulfate/Acetic acid 0.1%w/w / 0.5%w/w / 2%w/w Ear Spray
Legal category	POM
Storage	Store below 25°C. Do not refrigerate or freeze. Store upright.

Route/method of administration	Topical spray into the external auditory canal. Before first use, press actuator down several times to obtain a fine spray. Then administer spray directly by gently placing nozzle tip into ear opening and pressing down once on the actuator. Each press then delivers one metered dose. Do not inhale the spray.
Dose and frequency	One metered dose (60mg) into the affected ear three times daily.
Quantity to be administered and/or supplied	One 5 mL bottle per treatment course.
Maximum or minimum treatment period	Typically 7 days; reassess if symptoms persist. Treatment should ideally be continued until two days after symptoms have disappeared
Adverse effects	Local irritation, stinging or burning sensation. Rare: allergic reaction, skin sensitisation, or superinfection. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 7 days, or they become systemically very unwell.

	Provide advice on sources of information and support. Provide advice on self-care measures for symptom relief and to reduce risk of recurrent infection. Advise on options for analgesia for symptom relief, if needed.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
1/11/25		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		3/11/2025
Chris Pilkington	Pharmacist GPhC: 2046322		23/10/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	23 rd Oct 2025